



Hospital Registration Number

Demographics

Service Name Lab Practice EMS	Service No. 100	CACC/ACS 921	Call Number 2026-0912-0233	Call Date 2026/09/12	
Last Name Holloway			First Name Dennis		
Age 64 Y	Sex M	Weight (kg) 92	Date of Birth 1962/04/18	Health Insurance Number 0000-000-000	Version ZZ

Mailing Address

Street No. 48	Street Name Mill Street	City/Town Riverside	Province ON	Postal Code K0L 0A0	Country Canada
Pick-up Location or Sending Facility (City/Town) Riverside			☒ Same as Mailing Address Above		Pick-up Code R

Clinical Information

Date of Occurrence 2026/09/12	Time of Occurrence 06:40	Chief Complaint Central chest pressure x 50 min, radiating to L arm	☐ Positive for FREI
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Incident History

Wife called 911. Pt woke at 0640 with central chest pressure radiating to his L arm, 7/10, with nausea and mild SOB. Not relieved by rest. Pt says it feels like his angina but has lasted much longer. He did not use his own nitro spray. Denies trauma, fever, cough, recent illness or recent travel. No syncope.

MOHLTC DNR Confirmation Number

	Trauma Location	Problem Type	Site/Type Mechanism
1			
2			
3			

Relevant Past History

Provided by: ☒ Patient ☐ Other: _____

☐ Previously Healthy ☒ Cardiac ☐ Stroke/TIA ☐ Seizure ☐ Psychiatric ☐ Cancer ☐ CNO
☐ Respiratory ☒ Hypertension ☐ Diabetes ☐ Anaphylaxis ☐ Other (list below)

Details Stable angina (2023), hypertension, high cholesterol. No previous MI. Provided by pt, confirmed by wife.

Medications

☐ None	☒ Nitrates	☐ Insulin/Oral Diabetic Meds	☐ Phosphodiesterase Inhibitors
☐ CNO	☒ ASA	☐ Blood thinner/Anticoagulants	☐ Salbutamol ☐ Furosemide

Other Ramipril 10 mg daily, atorvastatin 40 mg daily, ASA 81 mg daily (last dose last night), nitro spray PRN (none today). Denies PDE5 inhibitor use.

Allergies

☒ NKA ☐ CNO ☐ Other – list below

Details

Treatment Prior to Arrival

☒ None ☐ EFRT ☐ Physician ☐ Fire ☐ Bystander ☐ CNO
☐ Midwife ☐ Other Paramedic ☐ Nurse ☐ Police ☐ Self ☐ Other (list below)

Details

Cardiac Arrest Information

	Date	Start Time
Arrest Witnessed By ☐ Bystander ☐ Trained Responder ☐ Paramedic ☐ Unwitnessed		
CPR Started By ☐ Bystander ☐ Trained Responder ☐ Paramedic ☐ None		
First Shock By ☐ Bystander ☐ Trained Responder ☐ Paramedic		

Physical Exam

General Appearance Skin Colour Pale Skin Condition Diaphoretic

Sitting upright on edge of bed, alert and oriented, pale and diaphoretic, anxious, speaking full sentences.

Head/Neck Trachea ☒ - Midline Shifted ☐ - R ☐ - L JVD ☐ - Elevated ☒ - Not Elevated
No JVD at 45 degrees.

Chest Air Entry ☒ - Bilaterally Decreased ☐ - R ☐ - L Breath Sounds ☒ - Clear ☐ - Wheezes ☐ - Crackles ☐ - Rub ☐ - Absent
Equal chest rise, no accessory muscle use. Chest wall not tender to palpation. Clear to bases bilaterally.

Abdomen ☒ - Soft ☐ - Rigid ☐ - Distended ☐ - Tender ☐ - Mass ☐ - Pulsatile ☐ - RU ☐ - LU ☐ - LL ☐ - RL ☐ - Center
Soft, non-tender all quadrants.

Back/Pelvis ☒ - Unremarkable

Extremities ☐ - Unremarkable Peripheral Edema ☒ - Absent ☐ - Present Pedal Pulse ☐ - Absent ☒ - Present
Radial pulses strong and equal bilaterally. No calf tenderness or swelling.

Practice tools

Check my ACR

Clear highlights

Read back

Review page

Lock for submission

Unlock (instructor)

How this form works

	Remarks
	Pt found sitting on edge of bed, pale and diaphoretic, hand on chest. Pressure is central, 7/10, spreading to L arm, started at rest at 0640, not changed by position or breathing. Similar to his usual angina but longer, and it has not settled. Mild nausea and SOB, no vomiting. 12-lead at 0737: sinus rhythm, no STEMI criteria, so transported to closest ED (Fleming General) rather than a PCI centre. ASA 160 mg chewed after confirming no allergy, no active bleeding and no asthma. He takes 81 mg daily, last dose last night. Asked specifically about erectile dysfunction medications (PDE5 inhibitors): pt denies. NTG 0.4 mg SL x2, BP checked before each dose, pain 7/10 to 5/10 to 3/10. BP remained above 130 systolic. Repeat 12-lead en route unchanged. Pt less diaphoretic by arrival, still pale. Verbal report to triage RN (K. Singh), care transferred at 0831. Wife followed in own vehicle with his wallet and medications.

Clear highlights

Disposition of Effects ☐ Receiving Staff ☒ Family ☐ Other (list):			
Primary Problem Cardiac - Ischemic (51)	Problem Code 51	Sp Trans Code	CTAS Arrive Patient 2
Deceased ☐ Obviously dead ☐ DNR ☐ BHP TOR ☐ Pronounced by on scene physician			CTAS Depart Scene 2
Physician/BHP Name (if pronounced/TOR)	Date	Time	CTAS Arrive Destination 2

7530-4714

Use when page 2 fills up. Rows continue from 27.

2026-0912-0233

Results

PRACTICE FORM - LAB USE ONLY - NOT A PATIENT RECORD

ACR Codes (1 of 2): Problem and Procedure Codes

Source: Ambulance Call Report codes, ontario.ca (page updated 2026/09/14). Codes change a few times a year: check ontario.ca/page/ambulance-call-report-codes.

Problem Codes			
VSA		116.01	Power Lifting Chair
1 Cardiac / Medical		117	Patient Lifting Device
2 Traumatic		117.01	Powered Lifting Device
Airway		231	Pt. Transported Supine
11 Obstruction (Partial/Complete)		232	Pt. Transported Semi-Prone
Breathing		233	Pt. Transported Prone
21 Dyspnea		234	Pt. Transported Semi-Sitting
24 Respiratory Arrest		235	Pt. Transported Sitting
Circulation		236	Ambulatory
31 Hemorrhage		237	Pt. Transported Lateral
32 Hypertension		238	Pt. Transported in Isolette
33 Hypotension		239	Infant Restraint Device
34 Suspected Sepsis		240	Knee Chest Position
Neurological		241	Exaggerated Sims Position
40 Traumatic Brain Injury		Suction and oxygen	
41 Stroke / TIA		120	Suction
42 Temp. Loss of Consciousness		120.1	Gastric Port Suctioning
43 Altered Level of Consciousness		129	Oxygen - Filtered High Conc. Mask
44 Headache		130	Oxygen - High Conc. Mask
45 Behaviour / Psychiatric		131	Oxygen - Simple Face Mask
45.01 Excited Delirium		132	Oxygen - Nasal Cannula
46 Active Seizure		133	Oxygen - Other
47 Paralysis / Spinal Trauma		141	Oxygen - BVM
48 Confusion / Disorientation		141.1	Manual Ventilation Feedback Device
49 Unconscious		142	Oxygen - Mechanical Ventilator
50 Post-ictal		144	Oxygen - Pocket Mask
Cardiac		Obstetrical	
51 Ischemic		150	Extricate Patient
53 Palpitations		160	OB Delivery
54 Pulmonary Edema		160.01	Delivery of Head
55 Post Arrest		160.02	Delivery of Umbilicus
56 Cardiogenic Shock		161	External Uterine Massage
57 STEMI		161.01	External Bimanual Compression
58 Hyperkalemia		162	Placental Delivery
Non-Traumatic		163	Clamp and Cut Umbilical Cord
60 Non Ischemic Chest Pain		164	Apgar Score
61 Abdominal / Pelvic / Perineal / Rectal Pain		165	Nuchal Cord
61.1 Renal Colic		165.01	Removal of Nuchal Cord
61.2 Suspected UTI		166	Shoulder Dystocia
62 Back Pain		166.01	McRoberts Maneuver
Gastrointestinal		166.02	Apply Suprapubic Pressure
63 Nausea / Vomiting / Diarrhea		166.03	Gaskin Maneuver
Integumentary		167	Breech Presentation
65 Integumentary		167.01	Manual Release of Limbs
65.1 Skin Tear		167.02	Mauriceau Smellie Veit Manoeuver
Musculoskeletal / Trauma		168	Prolapsed Cord Management
66 Musculoskeletal		Airway and breathing	
67 Trauma / Injury		170	Oro / Nasopharyngeal Airway
Obstetrical / Gynecological		180	Restrain Patient - Physical
71 Obstetrical Emergency		181	Spit Hood Application
72 Gynecological Emergency		190	Abdo / Chest / Back Thrusts
73 Newborn / Neonatal		211	Symptom Assist Medication
Endocrine / Toxicological		318	Supraglottic/Alternate Airway
81 Drug / Alcohol Overdose		319	Supraglottic/Alternate Airway Unsuccessful
81.1 Suspected Opioid Overdose		320	Needle Thoracostomy
81.2 Alcohol Intoxication		321	Needle Thoracostomy Unsuccessful
81.3 Drug Overdose		322	Needle / Surgical Cricothyroidotomy
82 Poisoning / Toxic Exposure		323	Needle / Surgical Cricothyroidotomy Unsuccessful
83 Diabetic Emergency		324	Nasotracheal Intubation
84 Allergic Reaction		325	Nasotracheal Intubation Unsuccessful
85 Anaphylaxis		326	Orotracheal Intubation
86 Adrenal Crisis		327	Orotracheal Intubation Unsuccessful
General and Minor		328	ETT Suctioning
87 Novel Medications		329	Tracheostomy Tube Suctioning
88 Home Medical Technology		330	Tracheostomy Tube Reinsertion
89 Lift Assist		330.01	Tracheostomy Tube Reinsertion Unsuccessful
90 Inter-facility Transfer		331	Magill Forceps / Foreign Body Removal
91 Environmental Emergency		332	Magill Forceps / Foreign Body Removal Unsuccessful
92 Weakness / Dizziness / Unwell		333	Extubation - Any Advanced Airway (Intentional)
93 Treatment / Diagnosis & Return		334	Extubation - Any Advanced Airway (Unintentional)
94 Convalescent / Invalid / Return Home		335	Needle Thoracostomy One-way Valve Monitored
95 Infectious Disease		336	Respiratory System Eval. (ETCO2 and SAO2)
96 Organ Retrieval / Transfer		337	ETT Confirmation
98 Organ Recipient		338	SpO2
99 Other Medical / Trauma (see remarks)		339	PEEP
99.15 No Complaint		380	Alternative Airway
Procedure Codes		381	Alternative Airway - Unsuccessful
Assessment		382	Airway Adjunct / Bougie
1 Clinical Note		383	CPAP
10 Vital Signs		384	CPAP - unsuccessful
20 Patient Assessment		385	Oro/Nasopharyngeal Swab
20.01 LAMS Assessment Score		386	Peak Flow Meter Testing
20.02 Acute Stroke Protocol Assessment		Cardiac	
20.03 Clinical Opiate Withdrawal Scale (COWS)		297	Therapeutic Hypothermia
20.04 Point-of-Care Risk Assessment		298	Defibrillation - Pads On
20.05 Clinical Frailty Scale		298.01	Pads Placement AL
20.06 NEWS2 Scale		298.02	Pads Placement AP
25 Blood Sampling - Glucose Determination		299	Automated CPR Device
Wound care		300	CPR
100 Dressing		301	Rhythm Interpretation
100.01 Hemostatic Dressing		302	Cardioversion
100.02 Occlusive Dressing		303	Valsalva Maneuver
100.03 Post-Op Dressing		305	Defibrillation Dual Sequential
100.04 Povidone Iodine Dressing		306	Defibrillation - Manual
100.05 Tissue Adhesive		307	Defibrillation - Semi-Automated
100.06 Reinforced Skin Closure Strip		308	Analyze - SAED
100.07 Fixation Sheet		309	External Pacing
100.08 Non-Adherent Dressing		313	12-Lead Acquisition
101 Control Bleeding		313.1	15-Lead Acquisition
102 Arterial Tourniquet		314	Cardiac Monitor Disconnected
Immobilization		316	Return of Spontaneous Circulation
105 Immobilization - Head		317	Return of Spontaneous Resp.
109 Reduction		IV and access	
109.01 Patellar Reduction		340	IV Monitoring
110 Splint Other		341	IV Cannulation
110.01 Pelvic Binder Device		342	Lock
111 Cervical Collar		345	Normal Saline
112 Spinal Board		349	Other IV Solutions
113 Spinal Immobilization Extrication Device			
114 Traction Splint			
Lifting and positioning			
115 Adjustable Break Away Stretcher			
116 Lifting Chair			

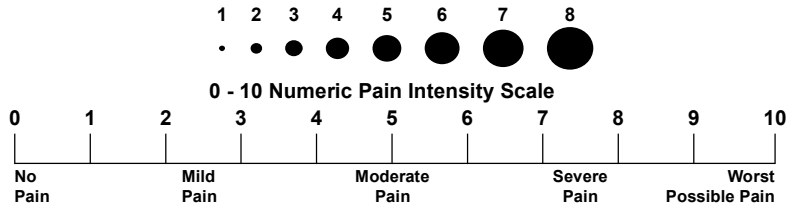
ACR Codes (2 of 2): Medications, Routes, Rhythms, Transport and Admin

Source: Ambulance Call Report codes, ontario.ca (page updated 2026/09/14). Codes change a few times a year: check ontario.ca/page/ambulance-call-report-codes.

Medication Codes		8	No STEMI Bypass - Pt. Condition	CTAS	Return priority can also be the CTAS level, 0 to 5
498	Acetaminophen	9	No STEMI Bypass - Hospital Refusal	No transport	
500	Adenosine	10	Palliative Care Pt. to Hospice/Hospital	8	Standby
502	Amiodarone	11	CMHA Middlesex Crisis Centre	9	Out of Service / Administration
503	Antibiotic	30	Specialized Team Transfer with Patient	71	No Patient Found
503.01	Amoxicillin	31	Specialized Team Transfer without Patient	72	Patient Refused
503.02	Cefuroxime	Trauma Site / Type / Mechanism		73	Patient Deceased
503.03	Trimethoprim/Sulfamethoxazole	A Location		74	Patient in Police Custody
503.04	Moxifloxacin	10	Head / Face / Ear / Scalp	75	Transported by Other Ambulance
503.05	Nitrofurantoin	11	Eye	77	Referred
503.06	Ceftriaxone	12	Neck	77.01	Referred - Palliative Care
503.07	Cefazolin	13	Shoulder	77.03	Referred - Mental Health and Addictions
503.08	Antibiotic - Ointment	14	Back / Flank	77.04	Referred - Non-Emergent
503.09	Cephalexin	15	Chest	77.05	Referred - Non-Emergent LTC
504	ASA	16	Abdomen	78	Discharge
505	Atropine	17	Pelvis	78.01	Discharged - Hypoglycemia
525	Calcium Gluconate	18	Genitourinary	78.02	Discharged - Seizure
528	Dextrose D10W	19	Buttocks / Perineum / Rectum	78.03	Discharged - Tachydysrhythmia
529	Dextrose D25W	20	Arm (Upper / Elbow / Forearm / Wrist)	78.04	Discharged - COVID-19
530	Dextrose D50W	21	Hand / Finger	78.05	Discharged - Mental Health and Addiction
531	Diazepam	22	Thigh	78.06	Discharged - Non-Emergent
533	Dimenhydrinate	23	Leg (Knee / Lower Leg / Ankle)	78.07	Discharged - Non-Emergent LTC
534	Diphenhydramine	24	Foot / Toes	Status Codes	
536	Dopamine	25	Hip	0	At Base
537	Dexamethasone	B Type		22	Receiving Facility
540	Epinephrine 1:1,000	30	Abrasion	44	On-Call
541	Epinephrine 1:10,000	31	Amputation	77	Mobile
550	Fentanyl	32	Avulsion	88	Standby Location
551	Furosemide	33	Burn	99	Maintenance
560	Glucagon	34	Blunt	Paramedic Designation	
561	Glucose - Oral	35	Crush	1	Student
562	Hydroxocobalamin	36	Contusion	2	EMA
565	Haloperidol	37	Penetrating / Perforation	3	PCP
593	Lidocaine	38	Possible Fracture / Dislocation	4	ACP
602	Ketamine	39	Laceration	5	CCP
603	Midazolam	40	Sprain / Strain		
604	Morphine	41	Paralysis / Paresthesia		
607	Hydromorphone	42	Other (Detail in Incident Hx)		
610	Naloxone	C Mechanism			
611	Buprenorphine/Naloxone	50	Assault		
615	Nitroglycerin	50.01	Assault - Physical		
620	Oxytocin	50.02	Assault - Sexual		
650	Salbutamol	50.03	Assault - Strangulation		
651	Sodium Bicarbonate	51	Drowning		
682	Xylometazoline	52	Electrocution		
700	Other Drugs - Detail in Procedures	53	Fall (Same Level)		
701	Anaesthetic Eye Drops	54	Fall from Height / Diving		
704	Ibuprofen	55	Gunshot		
706	Ketorolac	56	Hanging		
708	Obidoxime	57	Machinery		
710	Pralidoxime Chloride	58	MVC		
711	Hydrocortisone	59	Motorcycle / Recreational Vehicle		
712	Sodium Thiosulfate	60	Pedal Bicycle		
713	Glycopyrrolate	61	Pedestrian Struck		
728	Ondansetron	62	Sports		
729	Electrolyte - Potassium Elixir	63	Stabbing		
730	Lorazepam	64	Fire / Explosion / Thermal		
731	Immunization	65	Smoke / Chemical Exposure		
731.01	Tetanus	66	Other (detail in Mechanism of Incident Hx)		
732	Tranexamic Acid	Pick-up Codes			
900-999	User Defined	A	Airport / Heliport		
Routes of Administration		B	Apartment / Condo. Building		
AE	Aerosol	C	Construction Site		
BU	Buccal	D	Medical Office / Clinic		
ET	Endotracheal	E	Nursing Outpost		
IH	Inhaled	F	Factory / Industrial Site / Railway / Dockyard		
IM	Intramuscular	G	Hotel		
IN	Intranasal	H	Hospital (Acute and Non-Acute)		
IO	Intraosseous	I	Indoor Shopping Mall		
IV	Intravenous	J	Jail / Prison		
NB	Nebulized	K	Single Store / Strip Mall		
PO	Oral	L	School / College / University		
PR	Rectal	M	Mining Site / Quarry		
SC	Subcutaneous	N	Long-Term Care Home		
SL	Sublingual	O	Office Building		
TO	Topical	P	Sports Facility / Arena		
Rhythm Codes		Q	Farm		
10	Sinus Tachycardia	R	House / Town House		
11	PSVT / SVT / Atrial Tachycardia	S	Street / Highway / Road		
12	Atrial Flutter	T	Fairground / Park		
13	Atrial Fibrillation	U	Retirement Home		
14	Ventricular Tachycardia	V	Golf Course		
20	Sinus Bradycardia	W	Water / Boat		
21	First Degree Block	X	Restaurant / Bar		
22	Second Degree Block	Y	Casino		
23	Third Degree Block	Z	Other (Describe in Remarks)		
30	Ventricular Fibrillation	Za	Emergency / Transitional Shelter		
31	Pulseless Ventricular Tachycardia	Dispatch Priority			
32	PEA	1	Deferrable		
33	Asystole	2	Scheduled		
40	NSR	3	Prompt		
42	Paced Rhythm	4	Urgent		
43	Junctional Rhythm	8	Standby		
44	Sinus Dysrhythmia	9	Out of Service / Administration		
46	Other (Detail in Procedures)	P	MPDS Purple: emergent, immediately life-threatening		
CTAS		R	MPDS Red: emergent, potentially life-threatening		
0	Obviously Dead / TOR	O	MPDS Orange: urgent, potentially serious		
1	Resuscitation	Y	MPDS Yellow: non-urgent, not serious		
2	Emergent	G	MPDS Green: non-urgent, may be deferred		
3	Urgent	SBY	Incident Standby		
4	Less Urgent	EMCOV	Emergency Coverage		
5	Non Urgent	BSBY	Border Coverage		
Special Transport Codes		Return Priority			
1	Pt. Meets Trauma Criteria	1	Deferrable		
2	No Trauma Bypass - Pt. Condition	2	Scheduled		
3	No Trauma Bypass - Hospital Refusal	3	Prompt		
4	Pt. Meets Stroke Criteria	4	Urgent		
5	No Stroke Bypass - Pt. Condition	6	Transport of Deceased Patient		
6	No Stroke Bypass - Hospital Refusal				
7	Pt. Meets STEMI Criteria				

Reference Information

Skin	
Colour	Condition
Flushed	Dry
Pale	Clammy
Cyanosis	Diaphoretic
Jaundice	Unremarkable
Unremarkable	



Glasgow Coma Scale		
Eye Opening	Verbal Response	Motor Response
4 Spontaneous	5 Orientated	6 Obeys commands
3 To Voice	4 Confused	5 Localize (pain)
2 To Pain	3 Inappropriate words	4 Withdraw (pain)
1 None	2 Incomprehensible sounds	3 Flexion (pain)
	1 None	2 Extension (pain)
		1 None

Pediatric Coma Scale		
Eye Opening	Verbal Response	Motor Response
4 Spontaneous	5 Coos or babbles	6 Obeys commands
3 To Speech	4 Irritable & constantly cries	5 Withdraws from touch
2 To Pain	3 Cries to pain	4 Withdraws from pain
1 None	2 Moans to pain	3 Flexion to pain
	1 None	2 Extension to pain
		1 None

APGAR Score			
Parameter	0	1	2
Appearance	Blue or Pale	Pink body with blue extremities	Completely pink
Pulse (BPM)	0 (absent)	slow (<100)	≥100
Grimace Response	None	Some grimace	Good grimace
Activity and Muscle Tone	None, limp	Some flexion	Active, motion
Respiratory Effort	absent	<60 min	Good, crying
APGAR performed at 1 & 5 minutes after delivery Don't wait for APGAR to make decision on resuscitation			

Normal Pediatric Vital Signs		
Age	Respiratory Rate	Heart Rate
0 – 3 months	30 - 60	90 - 180
3 – 6 months	30 - 60	80 - 160
6 – 12 months	25 - 45	80 - 140
1 – 3 years	20 - 30	75 - 130
6 years	16 - 24	70 - 110
10 years	14 - 20	60 - 90
Systolic Blood Pressure (for children 1-10 yrs) ≥ 90 + (2x age in years) Weight (kg) = (age x 2) + 10		

Definition of Hypoglycemia

≥2 years Glucometry <4.0 mmol/L
<2 years Glucometry <3.0 mmol/L

Aid to Capacity Evaluation (Record Details in 'Remarks' Section)

Indicate to whom this assessment refers if not the patient [e.g., parent, or substitute decision maker (SDM)]

Patient verbalizes/communicates understanding of clinical situation? (e.g., what is wrong with you?) ☐ Yes ☐ No - Requires consideration of capacity

Patient verbalizes/communicates appreciation of applicable risks? (e.g., what could happen if I don't help you?) ☐ Yes ☐ No - Requires consideration of capacity

Patient verbalizes/communicates ability to make alternative plan for care? (e.g., what will you do once I leave?) ☐ Yes ☐ No - Requires consideration of capacity

Responsible adult on scene ☐ Yes ☐ No

Refusal of Service – I have been advised that treatment and/or transportation is available immediately. I refuse such treatment and/or transportation to hospital having been informed of the risks involved. I assume full responsibility arising out of such refusal.

Refus de service – On m'a avisé que je pouvais être traité ou transporté à l'hôpital immédiatement. Je refuse d'être traité ou transporté à l'hôpital. J'ai été informé des risques auxquels cette décision m'expose. J'assume l'entière responsabilité de ce refus.

Patient/ Substitute decision maker (SDM) – print name and address / Patient/mandataire spécial (MS) – Nom et adresse en lettres moulées

If SDM, relationship to Patient / Si MS, relation avec le patient

Time Date Signature of Patient or SDM / Signature du patient ou du MS

I have advised this patient or SDM of the risks to the patient's health that are involved.
J'ai avisé le patient ou le MS des risques de cette décision pour la santé du patient.

Time Date Attending Paramedic Signature

I was witness to the above-mentioned refusal and that the person has been informed of the risks involved.
J'ai été témoin du refus susmentionné et du fait que la personne a été informée des risques de ce refus.

Time Date Non Paramedic Witness Name Witness/Paramedic 2 Signature
Nom du témoin autre qu'un ambulancier paramédical Signature du témoin/d'un 2^e ambulancier paramédical

Documentation Review

2026-0912-0233

Student fills section A before submitting. The checker fills section C. The instructor uses section D.

A. Before you submit (student)

What was the one decision on this call that your chart most needs to explain? Does it explain it?

Going to the local ED instead of a PCI centre. The 12-lead result and that reasoning are in the Remarks, and the full-line row keeps the interpretation next to its time.

If you ran this call again, what would you chart differently, and why?

I would add the pain score to each NTG row instead of relying on the vitals rows around it. It would make the response easier to see at a glance.

B. Call summary (fills in from the chart)

Intervals

Crew notified to mobile: 1 min 25 s
Crew notified to arrive scene: 9 min 5 s
Arrive scene to patient contact: 2 min 50 s
On scene (arrive to depart): 24 min 20 s
Transport (depart to arrive destination): 24 min 15 s
Arrive destination to transfer of care: 11 min 15 s
Patient contact to first 12-lead: 3 min
Patient contact to first medication: 4 min
Call received to transfer of care: 69 min 20 s

Vitals trend

Sets of vitals: 5 (first 07:36, last 08:08)
HR: 96 > 92 > 88 > 84 > 82
BP: 158/94 > 154/92 > 146/88 > 138/84 > 136/82
RR: 20 > 18 > 18 > 16 > 16
SpO2: 95 > 96 > 96 > 97 > 97
GCS: 15 > 15 > 15
Pain: 7 > 7 > 5 > 3 > 3
Shock index, first set (HR / systolic): 0.61

These numbers are for debrief, not grading. Interval targets depend on the call and your service. Ask what drove each one, and whether the chart would let someone else work it out.

C. Check my ACR results (filled by the checker)

Checked 09:04

No documentation gaps found by the checker. That doesn't mean the chart is good. It means nothing obvious is missing.

Questions the checker can't answer for you:

- It took 3 min to reach the patient and you were on scene 24 min. If either delay was considerable, the reason belongs in Remarks (ODS 4.0). Is it there?
- Is the onset and character of the pain recorded (OPQRST), including what the patient was doing when it started?
- Is there a 12-lead time and your interpretation, and does the chart show whether it changed where you went or what you did?
- If ASA or nitro was given, are the checks you made first on the chart (allergies, recent PDE5 inhibitor use, BP before each nitro)?
- If you held a medication back, does the chart say why?
- Is the patient's response after each dose recorded, with a pain score and vitals?
- Does the Incident History say how and where you found the patient, and who told you what (ACR Manual)?
- Is your Primary Problem the most likely underlying cause, not the chief complaint restated? Shortness of breath can be the complaint and pulmonary edema the problem (ACR Manual).
- Could someone who wasn't on the call follow what happened from your Remarks alone?
- Is there something you decided not to do? The standards expect the reason for delaying or withholding an indicated treatment on the chart (ODS 4.0).
- If the patient declined any assessment or treatment, is it in the grid with their reason? If you had concerns the patient is at risk, are they in Remarks and in your handover (ODS 4.0)?

D. Instructor notes

How this practice ACR works

What this is

A practice version of the Ontario Ambulance Call Report (form 1881-45) for lab and simulation. It is not a patient record. It behaves like a careful paper chart, with a few extra tools that only work on screen and never print.

Use Adobe Acrobat Reader

The tools, checks and buttons need Acrobat Reader (free, desktop or iPad). In a browser or Mac Preview you can still type, but nothing will check or calculate. Save often.

The treatment grid

Vitals go in their columns, and so does a medication: the code, then the dose and unit, then the route. For a procedure, the ACR Manual has you write what was done in the columns next to the code, and the result across the Results columns. The two small boxes in the left margin do that. P writes across the procedure side (Dose/Unit to Temp). R writes across the Results side (Reading/Code to Pain Scale). Tick one or both. For example, 341, then P: "20ga L forearm" and R: "Successful and secured". A medication can be written this way too: P: "160 mg PO", R: "Chewed and swallowed". Untick a box to go back to its columns. If the text runs out of room, tick the same box on the next row and carry on. Page 3 is a continuation page with 26 more rows and extra Remarks. Page numbers update on their own.

Codes

Pages 4 and 5 list the current Ontario ACR codes, taken from ontario.ca (page updated 2026/09/14). Codes are written the way Ontario lists them: 10 for vital signs, and decimal sub-codes where they exist, such as 401.4 for a STEMI notification. The code list changes a few times a year, so check ontario.ca/page/ambulance-call-report-codes if something looks missing.

Checks as you type

Dates must be YYYY/MM/DD and times 24-hour (1952 becomes 19:52). Procedure codes are checked against the current code list, doses need a unit, and routes need a route code. Choosing a Primary Problem fills in the Problem Code. Ticking one option clears any option it contradicts.

Check my ACR

Finds documentation gaps: blank required fields, times out of order, procedures with no time or crew number, medications with no dose, route, or vitals before and after, CNO with no explanation, transfer of care with no receiving person, and places where the chart contradicts itself. Items marked (ODS 4.0) come from the Ontario Ambulance Documentation Standards. It highlights fields in yellow on screen and lists them, then asks questions it can't answer for you. The highlights clear before printing. A clean check means nothing obvious is missing. It doesn't mean the chart is good.

Read back

Builds a transfer of care report from what is on your chart. Say it out loud. If something you would want to tell the receiving nurse isn't in the read-back, it isn't in your chart.

Corrections

Once a chart is submitted, don't change the original entries. Add an addendum at the end of Remarks, clearly marked as an addendum, with the date and time you added it. This is how the Documentation Standards require corrections on a real call report.

Submitting

Fill in the reflection on the Documentation Review page, run Check my ACR, then press Lock for submission. Everything except the review pages becomes read-only and page 1 is stamped. Your instructor can unlock it.

Printing

Print pages 1 and 2, page 3 if you used it, and page 6 if there was a refusal. Tools, highlights and margin boxes don't print.

Where the rules come from

Checker items tagged (ODS 4.0) come from the Ontario Ambulance Documentation Standards. Items tagged (ACR Manual) come from the Ambulance Call Report Completion Manual, and most field tooltips quote its instructions for that field. Where the 2017 manual and the current code list disagree (for example special transport codes written 01 or 1, or the newer status codes), the form follows the current ontario.ca code list.

References

Ontario Ambulance Documentation Standards, version 4.0, effective 2025/09/02.

ontario.ca/document/patient-care-documents-reference/ontario-ambulance-documentation-standards

Ambulance Call Report codes, ontario.ca, page updated 2026/09/14. ontario.ca/page/ambulance-call-report-codes

Ambulance Call Report Completion Manual, version 3.0, in force 2017/04/01 (Ministry of Health and Long-Term Care). The field-by-field guide to the ACR.

Page map

- 1 Demographics, history and physical exam
- 2 Treatment grid, remarks, disposition and crew
- 3 Treatment continuation (only if needed)
- 4 Codes: problem and procedure
- 5 Codes: medications, routes, rhythms, transport and admin
- 6 Reference tables, capacity and refusal
- 7 Documentation Review (reflection, call summary, checker results, instructor notes)
- 8 This page